TSHR and IGF1R in Graves disease and thyroid eye disease: genetic evidence from blood expression

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Abstract

Objective: Thyroid eye disease (TED) can accompany Graves disease (GD), an autoimmune cause of hyperthyroidism. We compared genetic evidence linking blood *TSHR* and *IGF1R* expression with susceptibility.

Methods: We screened 2,544 druggable genes using Mendelian randomization (MR). Disease outcomes were Biobank Japan (BBJ) GD, UK Biobank (UKB) hyperthyroidism and overlapping FinnGen GD, Graves ophthalmopathy (GO) and strict GO endpoints. Colocalization tested 24 genes for a shared causal variant. Tissue eQTL and orbital RNA-sequencing (four TED, one control) were supplementary.

Results: Thirteen genes passed BBJ discovery ($P < 0.05/2{,}544$). Higher genetically proxied *TSHR* expression was associated with lower GD odds in BBJ (odds ratio [OR] per approximately one standard deviation 0.12, 95% confidence interval [CI] 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$), concordantly across all five datasets. Shared-variant probability exceeded 0.80 in FinnGen GO and, prior-sensitively, BBJ and strict GO, but UKB (0.226) and FinnGen GD (0.264) favored distinct variants. *IGF1R* ORs exceeded 1 but reached $P < 0.05$ only in BBJ (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB ($P = 0.0117$); the most precise estimate, FinnGen GD, was OR 1.07 (95% CI 0.86–1.33). Single-variant tissue estimates added no independent evidence. Mean orbital *TSHR* and *IGF1R* transcript levels exceeded the control.

Conclusions: Association evidence was stronger for *TSHR* than *IGF1R*, but absent colocalization in UKB and FinnGen GD leaves expression-mediated causality unproven. Population-controlled data cannot isolate TED susceptibility within GD; weaker *IGF1R* evidence does not argue against IGF-1R–targeted treatment.

Keywords: Graves disease; thyroid eye disease; *TSHR*; *IGF1R*; Mendelian randomization; colocalization

Introduction

Thyroid eye disease (TED) can cause proptosis, diplopia and visual impairment in patients with Graves disease (GD) [1, 2]. Teprotumumab, which blocks the insulin-like growth factor 1 receptor (IGF-1R), improves clinical outcomes in active TED [3]. Its efficacy raises a related but distinct question: does inherited variation influencing *IGF1R* expression contribute to disease susceptibility in the same way as variation at *TSHR*, the established GD autoantigen and susceptibility locus?

TSHR and IGF-1R interact in experimental models of orbital disease [4, 5, 6]. However, treatment response, inherited gene regulation and gene expression in inflamed tissue address different biological questions. A receptor may be a useful treatment target without equally strong genetic evidence linking its expression to disease onset. Distinguishing these forms of evidence helps clinicians interpret genetic studies of therapeutic targets.

Mendelian randomization (MR) uses variants associated with an exposure to investigate its relationship with the disease phenotype being studied, termed the outcome [7]. Here, the exposure is blood gene expression and the outcomes are disease diagnoses, not responses to treatment. Because alleles are allocated at conception, MR is less prone than ordinary observational studies to confounding and reverse causation, but it supports causal interpretation only when its instrumental-variable assumptions hold. Colocalization asks whether expression and disease associations in a genomic region are consistent with the same causal variant [8]. Distinct nearby variants can appear connected because they are inherited together. Thus, an MR association may be highly significant even when the regional signals do not support a shared variant. MR reports odds ratios (ORs), confidence intervals (CIs) and *P* values; colocalization reports model-based posterior probabilities, which are not *P* values.

Previous genetic studies have already implicated *TSHR* in GD and Graves ophthalmopathy. Ji and colleagues integrated blood methylation, expression and protein associations with GD, reporting *TSHR* among the expression-associated genes also supported in a FinnGen ophthalmopathy dataset [9]. Other MR work has examined inflammatory proteins in ophthalmopathy [10], and recent integrative research has investigated *TSHR* across genetic and transcriptomic data in GD and ophthalmopathy [11]. These findings establish a basis for comparing clinically relevant receptors, but do not determine whether evidence for their blood expression is consistently supported across disease definitions under the same analysis framework. In particular, limited evidence for *IGF1R* could reflect imprecision or dependence on particular instruments, while a strong MR association for *TSHR* does not by itself establish that expression and disease share a causal variant. The remaining question is how robustly these two expression traits are associated with GD and related phenotypes, and how far those associations support biological interpretation.

To address this question, we compared genetically proxied blood expression of *TSHR* and *IGF1R* — expression predicted from inherited variants rather than measured directly — within a screen of genes encoding potentially druggable proteins, using *CTLA4* and other associated genes as context. We used Biobank Japan (BBJ) GD for discovery, compared the original findings with UK Biobank (UKB) hyperthyroidism and FinnGen ophthalmopathy, and subsequently added FinnGen GD and a stricter ophthalmopathy definition. We evaluated effect direction, precision and statistical support across these datasets, then examined dependence on individual instruments, allele frequencies and colocalization assumptions. A later fixed-panel tissue extension evaluated whether these patterns extended beyond the original blood instruments. Our aim was to characterize the consistency and limits of expression-related genetic evidence for the two receptors. The analyses distinguish GD from broader hyperthyroidism and population-controlled ophthalmopathy; they do not directly

estimate TED susceptibility among patients who already have GD or predict treatment response. A small in-house orbital RNA-sequencing series (four TED samples and one control) is reported only as descriptive context.

Methods

Study design and disease datasets

This two-sample MR study used public association summary statistics and followed STROBE-MR reporting guidance [12]. The exposure used whole-blood expression quantitative trait locus (eQTL) statistics from eQTLGen, comprising up to 31,684 participants, predominantly of European ancestry [13]. The original outcome hierarchy comprised BBJ GD, UKB self-reported hyperthyroidism or thyrotoxicosis, and FinnGen R12 Graves ophthalmopathy [14, 15, 16]. We subsequently added FinnGen R12 E4_GRAVES_STRICT and E4_GRAVES_OPHT_STRICT. The five datasets and their case and control counts are shown in Table 1. The UKB source did not establish an exclusively European analysis sample, so ancestry was not inferred from the reference panel used in its source study.

BBJ remained the discovery dataset. FinnGen GD provided a same-disease comparison, UKB a broader-phenotype comparison, and the two ophthalmopathy endpoints sensitivity analyses of a TED-enriched phenotype. The three FinnGen endpoints contain overlapping participants and were not treated as independent replications or combined by meta-analysis. Their controls were population non-cases for the respective endpoint, not GD patients without TED. The FinnGen extensions, expanded candidate assessment and instrument diagnostics were post hoc; no prospective registration was available.

Expression instruments and MR

From 4,462 potentially druggable genes [17], instruments were selected from significant blood cis-eQTLs ($P < 5\times10^{-8}$) within 1 Mb of the gene. PLINK clumping used a linkage disequilibrium (LD) threshold of $r^2 < 0.001$ within 10 Mb and 503 European participants from 1000 Genomes Phase 3 [18, 19]. This produced 6,135 single-nucleotide polymorphism (SNP)–gene records for 2,544 genes. The minimum F statistic was 29.7; the median variant-specific expression sample size was 28,092, with a range of 232–31,684. Exposure coefficients and standard errors were reconstructed from Z statistics, sample sizes and reference allele frequencies under unit expression variance [20]. Alleles were aligned with TwoSampleMR; ambiguous palindromic or unavailable variants were excluded. UKB linear-model coefficients and standard errors were divided by $\mu(1-\mu)$, where $\mu = 3{,}731/484{,}598$, to approximate a log-odds scale. BBJ and FinnGen coefficients already used log odds.

We used Wald ratios for single-instrument genes and multiplicative random-effects inverse-variance weighting (IVW), with correction for under-dispersion, for multiple instruments [21]. An instrumental variable (IV) is a genetic variant used to represent variation in expression. Results are reported as ORs per approximately one standard deviation higher blood expression, with 95% CIs and two-sided *P* values. This scale describes reconstructed expression effects, not a drug dose or an intervention. Primary discovery required $P < 0.05/2{,}544$ (1.965×10^{-5}), retaining all eligible genes in the denominator even when an outcome estimate was unavailable. The 13 BBJ discoveries were assessed in FinnGen GD using a focused threshold of 0.05/13 and concordant effect direction. A separate full FinnGen screen used 0.05/2,544 and was exploratory. Nominal $P < 0.05$ was reported descriptively and was not equated with discovery significance.

MR interpretation assumes that instruments predict expression, are not associated with exposure–disease confounders, and influence disease only through the modeled exposure. A small *P* value or high PP.H4 cannot by itself establish these assumptions.

Where applicable, sensitivity assessment included Cochran's Q, alternative MR estimators and omission of individual instruments [22, 23]. Previously completed alternative-estimator results (MR-Egger, weighted median and weighted mode) for the original three outcomes were retained (Supplementary Dataset, MR_estimators_original); they were not represented as new analyses of every endpoint. Post hoc leave-one-out results assessed dependence on individual variants, not independent replication. Single-instrument *TSHR* results could not undergo these omission or multi-instrument pleiotropy tests.

In a post hoc allele-direction audit, we re-extracted rs179252 and rs179247 from the original expression and disease files. Disease effects were aligned to the blood expression-increasing allele, and G-allele correlations were calculated in European and East Asian reference genotypes and independently checked with PLINK. This audit assessed allele alignment and consistency with published thymus findings, rather than thymic mediation (Supplementary Dataset, TSHR_allele_audit).

Colocalization and robustness assessment

We report MR associations as the main findings and use colocalization to assess their interpretation. In the original analysis, colocalization was reported alongside MR as a parallel evidence layer; the present emphasis was adopted during post hoc reappraisal and was not a prospectively registered hierarchy. Colocalization used complete available regional cis-eQTL statistics, including non-significant variants, matched to each disease dataset [8]. The expanded set included all 13 BBJ discoveries, *IGF1R*, nine additional FinnGen GD discoveries and *HLA-G*, which was selected because its MR significance changed after

frequency substitution: 24 genes in total. We used coloc.abf with disease as dataset 1 and expression as dataset 2. Posterior probability of hypothesis 4 (PP.H4) measures support for a shared causal variant under the fitted model. PP.H3 instead supports distinct variants. We used PP.H4 ≥ 0.80 as a descriptive strong-support threshold, with priors $p1 = p2 = 10^{-4}$ and $p12 = 10^{-5}$, and repeated analyses at $p12 = 5\times10^{-6}$ and 10^{-6} [24]. All five hypotheses are defined in Supplementary Methods. Low PP.H4 alone does not establish no disease association, and H3 does not prove multiple independent signals.

We evaluated reference and eQTLGen cohort frequencies, distinguishing changes in effect scaling from changes in variant availability and harmonization. The 24-gene colocalization dataset contains 1,440 settings: 24 genes × five outcomes × four frequency/availability scenarios × three priors. For *TSHR*, we additionally examined narrower windows, a common five-outcome variant set, raw regional disease peaks and all available source cis-eQTLs. For newly evaluated candidates, we checked marginal signal coverage, shared instruments and variant aliases. *C4B* and *HLA-G* underwent instrument-level frequency, weighting, heterogeneity and omission checks. Full-precision results are supplied in one Supplementary Dataset workbook. The original BBJ–ophthalmopathy combined filter is retained as a historical descriptive comparison, not promoted to a prospectively specified validation rule or the primary conclusion of this reappraisal.

Tissue-specific extension

In a post hoc extension, we assessed both genes in a fixed panel of five GTEx v8 tissues: thyroid, cultured fibroblasts, subcutaneous adipose, visceral adipose and whole blood [25]. The extension plan was fixed before retrieving these tissue results, after the blood analysis was known. GTEx participants were predominantly of European ancestry, not exclusively European. Instruments required cis-eQTL $P < 5\times10^{-8}$, $F > 10$ and European-reference LD $r^2 < 0.001$. Ten planned gene–tissue comparisons against FinnGen GD formed the primary

family ($P < 0.005$); 40 comparisons against the other four datasets formed a secondary family ($P < 0.00125$). Unestimable combinations remained in the correction denominators. Effects used the original slopes and standard errors for inverse-normal-transformed expression. Complete regional summaries were not obtained for this extension, so tissue colocalization was not performed; significant-only pairs were used solely for instrument selection. Details and all planned combinations are in Table S3 and the Supplementary Dataset.

Orbital expression and implementation

An in-house orbital RNA-sequencing series of four patients with inactive TED and one control provided descriptive context only (Supplementary Methods). Technical replicates were combined by biological sample, and transcript abundance was displayed without differential-expression significance testing (Figure S1). These observations were not used to select genes or validate the genetic findings. Analyses used R 4.3.3, TwoSampleMR 0.7.4, coloc 5.2.3 and PLINK 1.9, with independent numerical verification in Python. No age- or sex-stratified patient analyses, individual-level multivariable regression, Steiger directionality test or multi-signal colocalization was performed.

Results

Discovery across druggable genes

Of 2,544 instrumented genes, 2,234 were estimable in BBJ, 2,505 in UKB and 2,480 in each FinnGen endpoint. Unavailable estimates were not counted as negative findings. Thirteen genes met the BBJ discovery threshold (Figure 1; Table 3A): *HLA-A*, *HLA-DQA2*, *CTLA4*, *TSHR*, *C4A*, *HSD3B7*, *TUBB*, *VKORC1*, *TNFSF14*, *PRSS36*, *MAPKAPK5*, *PSMB8* and *IFNGR1*.

TSHR and IGF1R across disease datasets

Higher genetically proxied blood *TSHR* expression was associated with lower odds of BBJ GD (OR 0.12, 95% CI 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen GD (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$). The same direction was observed for UKB hyperthyroidism (OR 0.09, 95% CI 0.06–0.14; $P = 8.77\times10^{-28}$), FinnGen ophthalmopathy (OR 0.10, 95% CI 0.04–0.24; $P = 2.82\times10^{-7}$) and strict ophthalmopathy (OR 0.15, 95% CI 0.06–0.39; $P = 9.02\times10^{-5}$). All five estimates used rs179252 as the single instrument (Table 2; Figure 2). The strict ophthalmopathy association reached $P < 0.05$ but not the full-screen threshold of 1.965×10^{-5}.

The expression-increasing rs179252-G allele was associated with lower odds in all five disease datasets and was positively correlated with rs179247-G in European (r = 0.978; r^2 = 0.957; n = 503) and East Asian (r = 0.996; r^2 = 0.992; n = 504) reference samples. The latter allele also increased blood *TSHR* expression. Allele-aligned recalculation reproduced the original MR estimates (Supplementary Dataset, TSHR_allele_audit).

All five *IGF1R* ORs exceeded 1, but no association met the full-screen threshold. BBJ GD (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB hyperthyroidism (OR 1.35, 95% CI 1.07–1.70; $P = 0.0117$) reached nominal significance, meaning uncorrected $P < 0.05$. The FinnGen estimates did not: GD OR 1.07 (95% CI 0.86–1.33; $P = 0.539$), ophthalmopathy OR 1.41 (95% CI 0.85–2.33; $P = 0.182$) and strict ophthalmopathy OR 1.52 (95% CI 0.92–2.49; $P = 0.0996$). The FinnGen GD estimate was the most precise and lay close to the null. Weighted-median estimates resembled the IVW results in BBJ (OR 1.56; $P = 0.0288$) and UKB (OR 1.42; $P = 0.00244$), whereas MR-Egger and weighted-mode estimates were imprecise ($P \geq 0.32$), and MR-Egger intercepts were non-significant (P = 0.66, 0.56 and 0.43 in BBJ, UKB and FinnGen ophthalmopathy), although these tests have little power with three or four instruments. These findings show directional consistency without statistically convincing confirmation in FinnGen. They do not establish absence of an association.

Comparison with other associated genes

Four genes passed the full-screen threshold in both GD datasets: *TSHR*, *CTLA4*, *HLA-DQA2* and *TUBB* (Table 3A). In FinnGen GD, *CTLA4* was associated with lower odds (OR 0.17, 95% CI 0.08–0.36; $P = 2.15\times10^{-6}$), whereas *HLA-DQA2* (OR 1.90, 95% CI 1.64–2.20; $P = 1.41\times10^{-17}$) and *TUBB* (OR 3.28, 95% CI 2.09–5.17; $P = 2.68\times10^{-7}$) were associated with higher odds. *HLA-DQA2* and *TUBB* lie in the major histocompatibility complex (MHC) region. *CTLA4*, an established autoimmune locus [26], and *TUBB* had direction-consistent nominal associations across all five datasets. *HLA-DQA2* also showed higher-odds associations in both ophthalmopathy endpoints, but the UKB estimate was non-significant and in the opposite direction.

Using the focused FinnGen GD threshold of $P < 0.05/13$, seven of the 13 BBJ discoveries had direction-consistent associations. The three additional genes were *C4A* ($P = 4.97\times10^{-5}$), *TNFSF14* ($P = 0.00361$) and *IFNGR1* ($P = 0.00330$). *VKORC1* was not estimable in FinnGen GD; the denominator remained 13. Although *HLA-A* had the smallest BBJ *P* value (2.60×10^{-23}), its FinnGen GD association was non-significant ($P = 0.648$). Thus, discovery significance alone did not determine consistency across datasets.

The full FinnGen GD screen identified 13 significant genes, nine outside the BBJ discovery set: *APOM*, *NEU1*, *IGLV2-18*, *IGLV3-12*, *TNF*, *HIST1H4E*, *LTA*, *LTB* and *C4B* (Table 3B). Several shared the same instrument: *APOM* and *NEU1* used rs3101018, and *IGLV2-18* and *IGLV3-12* used rs5759410. These pairs therefore do not represent independent genetic findings. Seven of the nine (*APOM*, *NEU1*, *TNF*, *HIST1H4E*, *LTA*, *LTB* and *C4B*) were instrumented within the extended MHC region on chromosome 6. Five MHC-region genes among the BBJ discoveries were also retained in follow-up; membership of this region was not an exclusion criterion.

Instrument and frequency sensitivity

The two-instrument *CTLA4* estimates were largely supported by rs13030124. Omitting it from FinnGen GD left an imprecise estimate ($P = 0.323$), consistent with the dependence previously observed in UKB and original FinnGen ophthalmopathy. Heterogeneity was present in FinnGen GD for *C4A* (Q-test $P = 1.27\times10^{-12}$), *TUBB* ($P = 3.67\times10^{-5}$), *CTLA4* ($P = 0.0275$) and *HLA-A* ($P = 0.0387$). For the first three, cross-dataset associations should therefore be distinguished from agreement among instruments. In BBJ and UKB, two of the four single-variant omissions removed nominal *IGF1R* significance: rs2654980 in both ($P =$ 0.0751 and 0.468), rs59467480 in BBJ ($P = 0.0503$) and rs117212126 in UKB ($P = 0.0529$); FinnGen estimates had not reached nominal significance. This indicates sensitivity to instrument removal, which can reflect changes in precision as well as the contribution of the omitted variant; it does not by itself identify an invalid instrument (Figure S2; Supplementary Dataset).

Frequency substitution preserved the original 13 BBJ discoveries and the principal *TSHR* and *IGF1R* interpretation (Supplementary Dataset, MR_original_freq and MR_new_freq), but not every exploratory FinnGen classification (Table 3C; Table S2). For *C4B* in FinnGen GD, the MR *P* value changed from 1.10×10^{-5} to 1.48×10^{-4}, crossing the full-screen threshold; both versions had Q-test $P < 10^{-6}$. For *HLA-G*, *P* changed from 0.107 to 1.72×10^{-5} as the IVW weight of rs150254595 increased from 21.3% to 58.9%. Omitting this variant left non-significant estimates under either frequency source. Independent calculations reproduced the eight full-set and 72 instrument-omission estimates for these two genes. Precision also varied across the screen. At the discovery threshold, 14.6% and 35.6% of estimable BBJ genes had 80% power to detect ORs of 1.5 and 2.0 under a normal-approximation benchmark; the corresponding FinnGen GD proportions were 24.8% and 52.2% (BBJ: Supplementary Dataset, Detection_original; FinnGen GD: Supplementary Methods).

Colocalization findings

Regional analyses tested whether the MR associations reflected a variant shared with expression (Figures 2 and 3; Table S1). For *TSHR*, PP.H4 was 0.951 in BBJ, 0.986 in FinnGen ophthalmopathy and 0.891 in strict ophthalmopathy under the primary prior. It was lower in UKB (0.226) and FinnGen GD (0.264), where PP.H3 was 0.774 and 0.736, respectively. The latter patterns favored distinct variants under the fitted model despite very small MR *P* values. This discordance persisted in narrower regions and a common five-outcome variant set. The matched analyses retained the regional disease peaks, and all three FinnGen comparisons shared 6,579 variants in the full cis-region. Lowering the shared-variant prior p12 from 10^{-5} to 10^{-6} reduced PP.H4 to 0.661 in BBJ, 0.875 in ophthalmopathy and 0.451 in strict ophthalmopathy.

IGF1R PP.H4 ranged from 0.010 to 0.404. In UKB, expression-only, distinct-variant and shared-variant probabilities were 0.400, 0.196 and 0.404, respectively. *CTLA4* PP.H4 was 0.201 in BBJ and 0.254 in FinnGen GD, compared with 0.953 in UKB and 0.978 and 0.979 in the two ophthalmopathy endpoints. Neither *HLA-DQA2* nor *TUBB* reached PP.H4 of 0.001 in any dataset. No selected gene reached PP.H4 ≥ 0.80 in FinnGen GD; this statement applies to the 24 genes assessed, not all 2,544 screened genes. *TNFSF14* and *IFNGR1* had strong BBJ support (0.994 and 0.989) without consistent support across the other definitions. For the two *IGLV* genes in BBJ, the absent lead expression variant and weak overlapping expression signals limited interpretation of low PP.H4. The nine additional FinnGen discoveries and sensitivity-selected *HLA-G* lacked strong support in every dataset. The chromosome 16p11.2 discoveries remained compatible with a shared regional association rather than three independent expression targets.

Tissue-specific extension

Four of ten gene–tissue combinations provided a qualifying instrument, each retaining one SNP after clumping (F = 32.1–61.1). Higher genetically proxied *TSHR* expression in subcutaneous and visceral adipose was associated with lower FinnGen GD odds (OR 0.36, 95% CI 0.31–0.43; $P = 1.28\times10^{-34}$ and OR 0.50, 95% CI 0.43–0.58; $P = 4.09\times10^{-19}$, respectively), with corrected support in BBJ and UKB. Subcutaneous estimates also passed correction in both ophthalmopathy datasets; visceral estimates did not ($P = 0.0368$ and 0.186). The two *TSHR* instruments, rs12101255 and rs74064796, lie 50 bp apart in *TSHR* intron 1, about 15 kb from rs179252 (European r^2 0.46 for rs12101255), and both are themselves blood *TSHR* eQTLs (eQTLGen $P = 4.1\times10^{-34}$ and 6.5×10^{-34}). With one instrument, each tissue *P* value equals that variant's disease-association z-test *P* value, so these estimates re-express the regional *TSHR* association rather than providing independent evidence. Neither thyroid nor cultured-fibroblast *IGF1R* estimates reached $P < 0.05$ in any dataset. Their FinnGen GD estimates were OR 0.76 (95% CI 0.56–1.02; $P = 0.0692$), opposite in direction to the blood estimates, and OR 1.11 (95% CI 0.89–1.38; $P = 0.359$). All 20 estimates and 30 unestimable comparisons are reported (Table S3; Supplementary Dataset, Tissue_extension).

Descriptive orbital expression

Mean orbital *TSHR*, *IGF1R* and *CTLA4* transcript levels in four TED samples exceeded those in the single control (Figure S1). With one control and no confirmatory expression testing, these observations do not establish differential expression or explain the genetic associations.

Discussion

This study addresses how consistently blood expression-related genetic evidence supports two clinically relevant receptors across GD and related disease definitions. *TSHR* showed associations with lower disease odds in both GD datasets, with concordant directions in

broader hyperthyroidism and ophthalmopathy. All *IGF1R* ORs exceeded 1, but only the BBJ and UKB estimates reached uncorrected $P < 0.05$, none met the discovery threshold, and the most precise estimate lay close to the null. Colocalization supported a shared variant for *TSHR* in BBJ and both ophthalmopathy endpoints but favored distinct variants in UKB and FinnGen GD, the two datasets with the most cases. The contribution is a comparison of the reproducibility and limitations of these associations within the same analytical framework. It does not establish a difference in biological importance between the receptors, and statistical significance in one gene but not another is not a formal test of different effects.

Our findings extend an existing body of genetic evidence. Ji and colleagues had already linked blood *TSHR* expression to GD in an earlier FinnGen release and reported support in FinnGen ophthalmopathy [9]; recent integrative work has also investigated *TSHR* in GD and ophthalmopathy [11]. We add a focused comparison with *IGF1R*, a GD comparison spanning BBJ and FinnGen, and an assessment of both genes alongside the wider druggable-gene screen. The result is consistent association evidence for *TSHR*, limited evidence for *IGF1R*, and an explicit distinction between strong MR associations and variable regional support for expression-mediated causality. This comparison uses shared public resources, including eQTLGen and FinnGen, so agreement with earlier work is not wholly independent replication. Different MR methods, FinnGen releases and colocalization settings also preclude interpreting differences between reports as direct contradictions. Our additional evidence concerns the consistency and sensitivity of the associations, rather than discovery of a new susceptibility locus.

The other associated genes provide useful context for this central comparison. *CTLA4*, *HLA-DQA2* and *TUBB* accompanied *TSHR* among genes meeting the full-screen threshold in both GD datasets. The multi-instrument European *CTLA4* estimates relied heavily on rs13030124; with that caveat, *CTLA4*'s direction-consistent associations across all five outcomes support

its use as an autoimmune comparator. *HLA-DQA2* was consistent across GD and ophthalmopathy definitions but not UKB hyperthyroidism, while *TUBB* and *C4A* showed substantial instrument heterogeneity. The strongest BBJ association, *HLA-A*, did not persist in FinnGen GD, illustrating why a smaller discovery *P* value does not necessarily identify a more transferable gene-specific association. Shared instruments and complex MHC linkage further prevent counting every associated expression trait as an independent causal target.

The lower-odds direction for *TSHR* need not contradict the pathogenic role of receptor-stimulating antibodies. Blood transcript abundance, tissue-specific expression and receptor activation are different exposures, and genetic regulatory effects can differ across tissues [25]. Our audit linked the blood expression-increasing rs179252-G allele to rs179247-G in both reference populations. The latter is the GD-protective allele associated with greater thymic transcription in published studies [27, 28] and also increased blood expression in eQTLGen. This directional consistency is compatible with a central-tolerance explanation, but the thymic step was not tested here, and this consistency does not demonstrate that thymic expression mediates the disease association. These linked variants are not independent replications or proven causal regulators. The marked departure of the MR ORs from 1 reflects scaling: rs179252 shifts reconstructed blood expression by only about 0.11 SD, and its per-allele disease ORs were 0.80 (BBJ), 0.77 (UKB), 0.78 (FinnGen ophthalmopathy), 0.77 (FinnGen GD) and 0.82 (strict ophthalmopathy). Dividing these moderate per-allele effects by the small expression effect yields the per-SD ORs, which should not be translated into the effect of experimentally increasing *TSHR* expression. The adipose instruments tag the same intron-1 region as rs179252 and are themselves blood eQTLs, so their concordant estimates re-express this regional association. They cannot address the distinct-variant pattern in UKB and FinnGen GD, establish an adipose-mediated mechanism or provide independent causal replication.

Experimental and clinical studies provide context for relating TSHR-directed immunity to orbital disease. TSHR can undergo cleavage and release its extracellular A-subunit. Expression of this subunit in an adenoviral immunization model induced greater thyroid-stimulating autoimmunity than a minimally cleaving receptor [29]; plasmid immunization subsequently reproduced orbital adipogenesis, fibrosis and muscle pathology in mice [30]. Daser and colleagues further reported enhanced inflammatory and hyaluronan responses to TNFα in orbital fibroblasts from TSHR-immunized mice [31]. Graves ophthalmopathy has also been documented in primarily euthyroid and hypothyroid patients [32], supporting a distinction between thyroid functional status and orbital immune activity. These observations make the ophthalmopathy associations biologically relevant, but they do not show that an A-subunit shed by human orbital fibroblasts, acting alone, causes TED independently of GD. Our instruments do not measure receptor cleavage or antibody activity. The ophthalmopathy comparisons use population controls and overlapping FinnGen endpoints, so neither their MR estimates nor their higher colocalization probabilities establish a TED-specific effect within GD or a difference between GD and TED.

For *IGF1R*, the defensible conclusion is that blood expression-related genetic evidence was limited in this analysis. Concordant ORs above 1 are worth reporting, but the two nominal associations and the imprecise ophthalmopathy estimates do not establish a confirmed susceptibility effect. Loss of nominal significance after omitting single variants further limits confidence in the original estimates, although instrument removal also reduces precision. With 80% power at $P < 0.05$, the FinnGen GD analysis could detect an *IGF1R* OR of about 1.36, so it constrains larger blood-expression effects but not modest ones. The fixed-panel extension found no nominal association for thyroid or cultured-fibroblast *IGF1R*, and directions were inconsistent across datasets. The confidence intervals, single-instrument design and lack of orbital eQTLs preclude excluding a clinically relevant effect. Cultured

fibroblasts are not orbital fibroblasts, and the unestimable tissue combinations are not negative findings. Weaker genetic evidence for *IGF1R* does not argue against IGF-1R blockade: teprotumumab efficacy rests on a randomized trial in active TED [3], and experimental TSHR–IGF-1R crosstalk [5] concerns receptor signaling rather than inherited susceptibility. The results do not demonstrate that *IGF1R* acts only downstream, or that an association would necessarily become detectable using orbital tissue. The small descriptive orbital series cannot resolve either question.

Colocalization does not replace the MR associations, but it constrains causal wording. Strong *TSHR* associations in UKB and FinnGen GD coexisted with H3-dominant regional patterns, and BBJ and strict ophthalmopathy support weakened under a lower shared-variant prior. Thus, association strength and evidence for a shared causal variant were not interchangeable. UKB and FinnGen GD had the most cases of the five datasets; their distinct-variant patterns could reflect finer resolution of a complex regional architecture rather than weaker association, but this was not tested. Ji and colleagues chose a shared-variant prior intended to allow weaker signals to colocalize and accepted PP.H4 above 0.50 [9], whereas our primary settings used $p12 = 10^{-5}$ and PP.H4 ≥ 0.80. These differences further discourage comparing binary colocalization labels across studies without considering the model settings. Single-causal-variant assumptions, ancestry differences and signal coverage can affect these posterior probabilities, particularly in complex immune regions. The regional checks excluded a simple missing-peak explanation for *TSHR* but did not establish the correct causal architecture. For *HLA-DQA2* and *TUBB*, strong MR findings without shared-variant support warrant reporting as associated signals rather than validated expression targets. Multi-signal analysis could clarify some discordance [33], but low PP.H4 neither erases a disease association nor by itself proves that the modeled expression has no causal role.

The sensitivity analyses identify which qualifications matter for the central comparison. Frequency substitution preserved the original BBJ discovery set, whereas *C4B* and *HLA-G* changed significance classifications through altered effect scaling and instrument weighting. Instrument omission identified concentrated evidence for *CTLA4* and reduced statistical support for *IGF1R*. These checks reuse the same data and do not supply independent replication. For researchers evaluating TSHR and IGF-1R, the added answer is that blood expression-related associations are more consistently supported for *TSHR*, while its tissue mechanism and the extent of an inherited *IGF1R* contribution remain unresolved. Independent expression instruments and tissue-relevant regulatory data would help address these uncertainties. Comparing GD patients with and without TED, while accounting for follow-up and selection into the GD group, would address orbital susceptibility directly. The present results inform interpretation and follow-up research, but do not guide treatment selection.

Limitations

The principal limitations involve the exposure and phenotype definitions. Predominantly European blood eQTL effects were applied to BBJ, requiring transferability of expression associations across ancestries. UKB self-reported hyperthyroidism is broader than GD, and an exclusively European source sample was not established. FinnGen's overlapping, population-controlled endpoints do not directly compare GD patients with and without TED; cohort differences and phenotype differences cannot be separated cleanly. Participant overlap with eQTLGen could not be quantified. Cohort-frequency replacement, which uses a release excluding Framingham participants, addresses only part of expression reconstruction. It does not establish correct tissue specificity, remove LD-reference limitations or independently validate rare-variant effects. The orbital series has a single control, no external replication and no inferential role. The GTEx extension had four estimable combinations, each with one

instrument, and predominantly European rather than ancestry-homogeneous exposure data. It reused the disease datasets and lacked tissue-specific regional colocalization.

The analytical limitations include post hoc selection, few instruments and a single-causal-variant colocalization model. *TSHR* has one retained European-reference instrument, precluding multi-instrument pleiotropy tests; several other genes have few variants or heterogeneous estimates. MHC analyses are particularly vulnerable to complex LD and correlated expression, and low PP.H4 can reflect model mismatch or limited signal coverage. Multi-signal colocalization, directionality testing and formal between-phenotype effect tests were not performed. Only 24 selected genes underwent colocalization; therefore its negative classifications cannot be generalized to the entire screen. Discovery and focused follow-up thresholds address specified families of tests, while the expanded scans and sensitivity analyses remain exploratory. Neither a non-significant MR result nor a posterior below 0.80 establishes absence of an effect.

Conclusions

Genetically proxied blood *TSHR* expression showed strong, directionally consistent associations with lower odds of GD and related phenotypes, whereas *IGF1R* evidence was weaker and imprecise. Colocalization supported a shared variant for *TSHR* in BBJ and the ophthalmopathy endpoints, partly prior-dependently, but not in UKB or FinnGen GD, so these data establish association rather than expression-mediated causality. Single-variant adipose instruments from the same locus added concordant estimates but not independent tissue evidence. Weaker genetic evidence for *IGF1R* does not argue against the therapeutic relevance of IGF-1R blockade. Experimental TSHR autoimmunity provides biological context for orbital involvement, but this study does not establish that pathway or isolate TED susceptibility within GD.

Declarations

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Conflict of interest. The authors declare that they have no conflict of interest.

Ethics approval. This study used publicly available, de-identified summary statistics and an institutionally approved orbital tissue dataset. The in-house orbital transcriptomic component was approved by the Institutional Review Board of Pusan National University Hospital (approval number 2104-018-102) and was conducted in accordance with the Declaration of Helsinki.

Informed consent. Written informed consent was obtained from all individual participants included in the in-house orbital tissue study.

Data availability. Instruments, primary MR and colocalization results, and sensitivity results are supplied as Supplementary Dataset. Analysis code is available from the corresponding author on reasonable request. Public summary statistics analyzed in this study are available from their original repositories: blood *cis*-eQTL data from the eQTLGen Consortium, tissue eQTL data from GTEx v8, the Biobank Japan Graves disease and UK Biobank hyperthyroidism genome-wide association statistics through the GWAS Catalog, and the FinnGen Release 12 GD and ophthalmopathy statistics from FinnGen. The in-house orbital RNA-seq data are available from the corresponding author on reasonable request, subject to institutional and ethical restrictions.

Author contributions. J.P. conceived and designed the study, performed the analyses, and drafted the manuscript. M.-S.K. contributed to data collection and interpretation. K.-H.S. and S.-W.Y. supervised the study and revised the manuscript.

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Figure legends

Figure 1. Study design and BBJ discovery. (A) Original blood-expression screen and subsequent disease-definition comparisons. FinnGen endpoints overlap and are not independent replications. (B) One point per estimable gene (n = 2,234), positioned by the GRCh37 coordinate of its strongest selected expression instrument. The horizontal line is P = 0.05/2,544, equivalent to $-\log10(P) \approx 4.707$. Thirteen discoveries and *IGF1R* are labelled. This is a gene-level MR plot, not a SNP GWAS Manhattan plot. The threshold adjusts the original discovery screen, not every later exploratory analysis.

Figure 2. *TSHR* and *IGF1R* across disease definitions, with *CTLA4* as comparator. (A) Wald/IVW odds ratios and 95% CIs for all 15 gene–dataset comparisons. The horizontal axis is logarithmic and the reference is OR = 1. Bold P values and an asterisk indicate nominal P < 0.05, not Bonferroni significance. ORs describe reconstructed blood expression, not a treatment intervention. (B) Aligned colocalization probabilities under p1 = p2 = 10^{-4} and p12 = 10^{-5}. H0/H1 denotes no association or disease association only; H2 expression only; H3 distinct variants; H4 a shared variant. Bold H4 numbers indicate ≥0.80. MR P values and posterior probabilities answer different questions.

Figure 3. Regional genetic evidence and *TSHR* prior sensitivity. (A) Primary PP.H4 for 24 selected genes and five datasets. Borders mark PP.H4 ≥ 0.80, not P-value significance. All lettering is black, and probabilities <0.001 retain their full precision in the workbook. Row dividers separate the three focal/comparator genes, the remaining BBJ discoveries, nine additional FinnGen GD discoveries, and sensitivity-selected *HLA-G*. Only these selected genes underwent colocalization. The low BBJ *IGLV* probabilities are limited by absence of the lead expression variant. (B) *TSHR* PP.H4 at p12 = 10^{-5}, 5×10^{-6} and 10^{-6}; p1 = p2 =

641 10^{-4}. The dashed line marks the reporting threshold of 0.80. FinnGen GO and GO strict are

642 overlapping ophthalmopathy endpoints; GD is E4_GRAVES_STRICT.